

Interview Transcript with Dr. Helena Brandt (Leipzig, Germany) Psychiatrist and Mental Health Researcher

Interviewer: Dr. Brandt, thank you for joining us. Could you start by telling us about your background and focus in psychiatry? Dr. Brandt: Certainly. I'm Dr. Helena Brandt, a psychiatrist and researcher at Leipzig University Medical Center. My work focuses on anxiety, depression, and trauma-related disorders. Before the pandemic, my primary research examined how

chronic stress impacts neurobiology – cortisol rhythms, inflammatory markers, things like that.

When COVID-19 began, our department found itself at the center of another kind of outbreak – a mental health crisis. The virus affected lungs, but the isolation, uncertainty, and fear affected minds everywhere.

Interviewer: How did your clinic adapt to the sudden surge in psychological distress? Dr.

Brandt: It was overwhelming. By April 2020, we saw a 60% increase in referrals – people with panic attacks, insomnia, obsessive hygiene rituals, or depression triggered by isolation. Even our regular patients, those with stable conditions, began relapsing.

We quickly shifted to telepsychiatry, setting up secure video consultations within two weeks. Initially, some patients were skeptical – they wanted face-to-face connection – but many later said it was a lifeline.

At the same time, we created an emergency hotline staffed by psychiatry residents and psychology interns, operating 24/7. It received nearly 10,000 calls in three months.

Interviewer: What were the main psychological patterns you observed in patients during the first year? Dr. Brandt: The first wave was dominated by acute anxiety – fear of infection, fear of dying alone. We saw a spike in obsessive-compulsive behaviors: people washing hands 50 times a day or disinfecting groceries obsessively.

The second phase, by late 2020, shifted toward depressive symptoms – hopelessness, fatigue, loss of purpose. The novelty of 'we're all in this together' faded. The third phase, post-Delta, brought existential distress – people questioning meaning, careers, even relationships.

What's remarkable is that the emotional curve of the population mirrored the epidemiological curve – hope rising and falling with case numbers.

Interviewer: How did healthcare workers themselves cope psychologically? Dr. Brandt: They were our hidden patients. Many doctors and nurses suffered what we now call moral injury – guilt from decisions they had to make, like triaging patients or restricting family visits. Some couldn't sleep, haunted by faces of patients who died alone.

Our hospital created peer-support groups and confidential counseling for medical staff. I personally led several group sessions where doctors cried openly for the first time in years.

It was powerful vulnerability became part of healing.

By mid-2021, burnout was epidemic. We started resilience workshops focusing on mindfulness, breathing, and small self-compassion rituals. But honestly, systemic change staffing, breaks, rest is the only sustainable prevention.

Interviewer: Were there specific populations more vulnerable than others? Dr. Brandt: Yes.

Three stood out: 1. Young adults, particularly students disrupted education and social lives led to identity crises. 2. Elderly individuals, isolated in care homes, often with limited digital literacy. Loneliness was lethal. 3. Women, especially caregivers balancing remote work, childcare, and family illness. Many reported burnout and depression.

Interestingly, we also saw increased domestic violence cases during lockdowns. Stress confined within households can turn toxic. Psychiatry had to collaborate with social services like never before.

Interviewer: How did the pandemic change psychiatry as a discipline in Germany? Dr. Brandt: Fundamentally. First, it normalized telemedicine. What was once a niche tool is now a permanent feature.

Second, it blurred the boundaries between mental and physical health. We saw firsthand how stress affects immunity and recovery mind and body are inseparable.

Third, it reframed public dialogue around mental health. In Germany, where stoicism is cultural, many people spoke openly about burnout and therapy for the first time. That cultural shift is historic and overdue.

Interviewer: Could you share any research findings your team published during this period? Dr. Brandt: Yes, we conducted a longitudinal study on 2,500 participants across Germany, tracking psychological well-being from 2020 to 2022. We found that: - Around 35% showed moderate to severe anxiety in the first year. - Sleep disorders increased by 40%. - People who maintained structured daily routines even simple ones like morning walks reported significantly better coping outcomes.

We also examined cortisol and cytokine profiles, showing that chronic psychological stress produced inflammatory patterns similar to metabolic disease. That's critical evidence for the psychoneuroimmunology link we've long theorized.

Interviewer: What was your experience with patients recovering from severe COVID illness? Dr.

Brandt: Many developed post-intensive-care syndrome nightmares, flashbacks, guilt for surviving when others didn't. Essentially, post-traumatic stress.

We started a special rehabilitation clinic integrating psychiatry, pulmonology, and physiotherapy. Patients would practice breathing exercises while discussing anxiety. Recovery was holistic body and mind healing together.

One patient told me, Every breath reminds me of what I survived. That line has stayed with me it captures the essence of trauma and gratitude intertwined.

Interviewer: How did children and adolescents fare psychologically during this time? Dr.

Brandt: They bore invisible scars. Adolescents experienced disrupted social development.

Depression and eating disorders rose dramatically. We saw children develop separation anxiety when returning to school they'd grown used to parental proximity.

Germany expanded child psychiatry capacity, but demand still exceeded supply. I believe the next decade will involve repairing the emotional development debt this generation incurred.

However, I also saw resilience children adapting, creating virtual friendships, learning empathy. We underestimate their capacity for hope.

Interviewer: How did you personally manage your own stress while treating others? Dr. Brandt:

It wasn't easy. Therapists are human. I often felt the same fear and fatigue my patients described. Meditation, long walks, and journaling helped. My colleagues and I kept a shared gratitude log small things like sunlight during rounds or a patient smiled.

One moment of grace was when a patient sent a handwritten note saying, Thank you for listening when no one else could. That letter still sits on my desk.

Therapists heal others, but sometimes, it's our patients who heal us.

Interviewer: How did Germany's broader mental health policy evolve in response? Dr. Brandt: The federal government launched several initiatives: - Funding for teletherapy platforms. - Grants for mental health research related to pandemic stress. - Campaigns to reduce stigma around seeking help.

But we still need more integration between psychiatry and primary care. Mental health must not remain a specialty silo it should be part of every medical conversation.

The next challenge is post-pandemic burnout society's collective exhaustion. The re-entry anxiety after lockdowns is real. Healing will take years, not months.

Interviewer: What gives you hope as we move forward? Dr. Brandt: The courage of patients. I've seen people confront their deepest fears and rebuild meaning from chaos. I've seen communities form support groups online and neighbors look after one another. Humanity showed its capacity for empathy.

And from a scientific standpoint, psychiatry is finally recognized as central to health, not peripheral. That acknowledgment that the mind is as vital as the heart or lungs is a lasting gift of this tragedy.

Interviewer: What personal lesson will you carry from this experience? Dr. Brandt: That compassion is the most advanced technology in medicine. Tools and drugs matter, but healing begins with presence. I've learned to listen more, speak less, and allow silence sometimes,

silence is what patients need most.

The pandemic stripped away the illusion of control, but it also revealed connection. None of us were truly alone just distanced. That realization changed how I see every patient now: not as a case, but as a fellow traveler through uncertainty.

Interviewer: Thank you, Dr. Brandt, for your empathy and insight. Dr. Brandt: Thank you. If these lessons endure, perhaps the suffering will not have been in vain.

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